February 20, 2026

Assistant Secretary Thomas Keane, MD

Assistant Secretary for Technology Policy / Office of the National Coordinator for Health Information Technology

Attention: HHS Health Sector AI RFI

Mary E. Switzer Building, Mail Stop: 7033A,
330 C Street SW
Washington, DC 20201.

CC: Steven Posnack, Principal Deputy Assistant Secretary for Technology Policy, ASTP/ONC

Re: **Request for Information: Accelerating the Adoption and Use of Artificial Intelligence as part of Clinical Care**

RIN 0955-AA13

Dear Assistant Secretary Keane,

This comment is by 1upHealth, a scalable serverless cloud data provider leveraging the power of the FHIR data standard to combine claims and clinical data for analytics and APIs to change behavior. We applaud the work of ASTP/ONC to enable modern APIs and power a digital health economy giving patients apps of their choice. We appreciate the ongoing work of ASTP/ONC in exploring how to make AI more practical for patient care.

General Points

AI relies on data and in particular good AI relies on good data. Without adequate data downstream HHS AI policies will be ineffective. HHS, in particular CMS payment policies, should focus on data fluidity and transparency and do so with robust privacy protections. As a practical matter this means incenting use of FHIR APIs and requiring public key cryptographic protections for the only parties with an actual right to patient data – namely the patient, the patient's providers and the patient's payer.

The fundamental challenge to get data for legitimate uses including AI is that CMS provider payment policies heavily incent making data NOT AVAILABLE whether for AI or any procompetitive purpose. Core to this is disproportionate payment for procedures so the dominant US business model has become that of an oligopoly provider using a single EHR where vast amounts of data are used internally and shared with other oligopoly customers but where external release of that same information to potential competition is heavily gated by cumbersome procedures, disproportionate costs, ancient protocols and at times outright throttling of data flows. This "information blocking" has been the pattern in healthcare for decades after widespread interoperability became the norm in every other sector of the economy.

As a start CMS payments should be made dependent on provider documentation being made available with modern RESTful APIs. The CMS standards group could modify X12 so that every provider claim includes the URL where documentation for the billed service becomes available to patient's, their apps (both AI and otherwise) as well as true HIPAA Covered Entities.

HHS could be the center of a modern digital healthcare economy serving patients. While HHS has historically supported FHIR APIs in the original 2020 Cures Act rule and with payer provided APIs under the CMS 0057 rule. Now HHS appears to support institutional information blocking in the form of TEFCA. The 2016 Cures Act defines information blocking as interfering with the "access, exchange and use" of patient data – TEFCA does all three. TEFCA's brokers interfere with data access, TEFCA's ancient AOL / Yahoo era protocols interfere with data exchange and TEFCA's document-only architecture interferes with data use. TEFCA proponents (not surprisingly incumbent data holders and those trying to sell patient data to third parties circumventing HIPAA) state that TEFCA will be moving to FHIR (the JSON based data standard). They have claimed this for the last 5 years yet TEFCA does not appear to have performed a single bona fide API transaction using FHIR.

Modern AI works best with massive amounts of data. TEFCA's antiquated architecture and baroque protocols are not up the task of moving the large amounts of data needed for robust AI. The cellphone Internet economy does billions of JSON transactions DAILY.

The Internet uses robust public key cryptography and "zero-trust" security for data needing protection. This is in stark contrast to TEFCA's "just trust me" faux-HIPAA "permitted uses". This end run around HIPAA is so brittle that TEFCA participants – the same QHINs which are supposed to vet other participants – are currently engaged in multiple lawsuits around their wholesale misappropriation of patient data. The data being resold here has almost invariably been subsidized by taxpayers.

Actual privacy and security should be the ground assumption for HHS AI work. RESTful and Bulk FHIR APIs anchored by public key cryptography can provide this level of privacy and security just as they do in banking and defense. HL7 has extensive implementation guides for healthcare versions of these APIs. HHS AI use should build on these APIs. Using such APIs CMS could analyze Medicare claims for fraud (matching actual diagnoses, plausible providers and appropriate services). MA, Medicaid and commercial plans could use similar tools and because working APIs are real-time this can be done before fraudulent claims are paid. Using the Bulk FHIR APIs which are batch mode population level APIs, CMS and other payers such as the VA and DOD could use AI to correlate patterns of care and best practices with outcomes and do so on a broad basis.

Today CMS measures quality with cumbersome data collection - QRDA for providers and Primary Source Verification for payers.

It is key for HHS to realize that enablement of ethical AI and patient-controlled digitally based competition are largely one and the same. Both rely on the same data and the same privacy security models. While one can see alternate data acquisition tools such as screen-scraping (robotic process automation) or any number of proprietary EHR vendor-gated strategies widespread use of AI in healthcare will require public APIs. Patients want apps which are smart, real-time, low cost and maximally effective. They do not want to be bound to only those services provided by incumbents and their “portals” or TEFCA’s convoluted IAS barriers.

1. What are the biggest barriers to private sector innovation in AI for health care and its adoption and use in clinical care?

The biggest barrier to the use of AI in healthcare is the reality that much if not most clinical data is held by a few EHR vendors and only available to whatever AI programs those vendors decide to allow. The vast bulk of data hidden here has been paid for by US taxpayers.

One major HHS policy would be to require that the EHR vendors patient-portals are required to run entirely through public APIs rather than hidden connections. Any patient data including scheduling which is not available at the same level of performance to outside competitive patient record products should be considered information-blocking. Cures Act enforcement should be initiated against both the EHR and the provider using that EHR. Having patient-controlled apps on a somewhat level playing field with provider / vendor-controlled apps would allow patients to get apps giving the patient their AI optimized medical advice and guidance the form patients as consumers decide on rather than being bound to their provider portal.

Another massive policy opportunity to open up data for AI is in how CMS pays Medicare Advantage plans. The Star ratings program and HEDIS measures use outdated data feeds. This data is not exposed to public APIs which AI vendors could use to improve the efficiency and cost-effectiveness care. One outgrowth of current Star rating policy are the so-called vendor Payer Platforms where EHR vendors use private APIs to help providers and payers game higher Star ratings. CMS should zero out payments for any relevant Star rating measures unless that data was obtained through a public API with each data element tagged with the metadata of that public API call. By requiring all Star ratings exclusively use public API data the entire quality measurement landscape could then be modernized by AI. This would have secondary public benefits of helping reform quality measurement and helping payers better negotiate on behalf of taxpayers and beneficiaries.

2. What regulatory, payment policy, or programmatic design changes should HHS prioritize to incentivize the effective use of AI in clinical care and why? What HHS regulations, policies, or programs could be revisited to augment your ability to develop or use AI in clinical care? Please provide specific changes and applicable Code of Federal Regulations citations.

The Cures Act requires that ALL data be made available. The 2020 ONC Cures Act rule predated widespread use of LLMs and had as its only requirement that each data field had a vocabulary tag of the vendor’s choice. ASTP should consider fleshing out the opportunities to more richly enable AI with this Cures Act requirement.

3. For non-medical devices, we understand that use of AI in clinical care may raise novel legal and implementation issues that challenge existing governance and accountability structures (e.g., relating to liability, indemnification, privacy, and security). What novel legal and implementation issues exist and what role, if any, should HHS play to help address them?

Requiring patient permission using modern cryptographic principles would be a solid foundation for AI data use.

8. Where would enhanced interoperability widen market opportunities, fuel research, and accelerate the development of AI for clinical care? Please consider specific data types, data standards, and benchmarking tools.

Requiring the large volumes of healthcare data needed for quality measurement and Star ratings to flow through public APIs rather than NCQA Primary Source Verification screen reviews or private payer provider platforms (which inevitably become anti-competitive) would be fundamental interoperability reforms. Incenting modern high-performance RESTful FHIR APIs rather than TEFCA’s brittle, expensive, slow and insecure document flows should be central to Federal policy.

Thank you for reviewing these comments.

Sincerely,

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